Q LABS DIAGNOSTICS

| | | | |
|---|---|---|---|
| Patient ID | : 10125/3380 | Sample ID | : 2512167 |
| **Name** | **: MRS M YADAMMA** | Registration Date | : 04/12/2025 07:54 AM |
| Age/Gender | : 71 Yrs/Female | Report Date | : 04/12/2025 05:24 PM |
| Ref. Dr. | : Dr. MD MUBASHEER ALI | 10125/3380 | |
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## HAEMATOLOGY REPORT

| Test Description | Result | Unit | Biological Reference Range |
|---|---|---|---|
| **COMPLETE BLOOD PICTURE** | | | |
| **Hemoglobin** | **10.8** | gm/dL | 11.0 - 15.0 |
| RBC Count | **3.39** | mil/cu.mm | 3.8 - 5.5 |
| Hematocrit HCT | **32.40** | % | 36 - 48 |
| **Platelet Count** | 2.88 | lac/cmm | 1.5 - 4.5 |
| **Red Cell Indices** | | | |
| Mean Corp Volume MCV | 95.6 | fL | 83 - 101 |
| Mean Corp Hb MCH | 31.9 | pg | 27 - 32 |
| Mean Corp Hb Conc MCHC | 33.3 | gm/dL | 31.5 - 34.5 |
| RDW-CV | 11.7 | % | 11.6 - 14.0 |
| **Total Count and DIFFERENTIAL LEUCOCYTE COUNT** | | | |
| Total WBC Count | 7450 | cell/cu.mm | 4000 - 10000 |
| Neutrophils | 73 | % | 40 - 75 |
| Lymphocytes | 22 | % | 20 - 40 |
| Eosinophils | 01 | % | 01 - 06 |
| Monocytes | 04 | % | 02 - 10 |
| Basophils | 00 | % | 00 - 02 |
| **Microscopic Blood Picture** | | | |
| RBC Morphology | **Anisocytosis Microcytic Normochromic** | | |
| WBC Morphology | With In Normal Limits | | |
| Platelet on smear | Adequate | | |

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H No. 8-1-284/OU/378, On Plot No.378, OU Colony, Manikonda, Hyderabad. www.qlabsdiagnostics.com Cell : 8520004903, 9063222152

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| | | | |
|---|---|---|---|
| **ESR - ERYTHROCYTE SEDIMENTATION RATE** | **38** | mm/hr | 0 - 22 |

*Method : Automated Westergren*

**INTERPRETATION :**

1. It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
2. It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
3. It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

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**HBA1C/GLYCOCYLATED**

| | | | |
|---|---|---|---|
| HbA1c Glycosilated Haemoglobin | **7.5** | % | 4 - 6 |

*Method: HPLC, NGSP certified*

| | | |
|---|---|---|
| Estimated Average Glucose : | 169 | mg/dL |

| As per American Diabetes Association (ADA) | |
|---|---|
| **Reference Group** | **HbA1c in %** |
| Non diabetic adults >=18 years | <5.7 |
| At risk (Prediabetes) | 5.7 - 6.4 |
| Diagnosing Diabetes | >= 6.5 |
| Therapeutic goals for glycemic control | Age > 19 years<br>Goal of therapy: < 7.0<br>Action suggested: > 8.0<br>Age < 19 years<br>Goal of therapy: <7.5 |

| ADA criteria for correlation | |
|---|---|
| **HbA1c(%)** | **Mean Plasma Glucose (mg/dL)** |
| 6 | 126 |
| 7 | 154 |
| 8 | 183 |
| 9 | 212 |
| 10 | 240 |
| 11 | 269 |
| 12 | 298 |

**Note:**
1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled .
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or
extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.
**Comments:**HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.

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## BIOCHEMISTRY REPORT

| Test Description | Result | Unit | Biological Reference Range |
|---|---|---|---|
| **KIDNEY FUNCTION TEST(KFT)** | | | |
| Serum Creatinine<br>*Method: Modified Jaffe with no deproteinzation* | 1.38 | mg/dL | 0.60 - 1.40 |
| Urea<br>*Method: Urease UV GLDH* | **46.76** | mg/dl | 10.0 - 45.0 |
| Uric Acid<br>*Method: Uricase Peroxidase* | 3.69 | mg/dl | 2.5 - 6.8 |
| Blood Urea Nitrogen-BUN<br>*Method: Calculated* | 18.7 | mg/dl | 7 - 20 |
| Sodium<br>*Method: ISE* | 144 | mEq/L | 135 - 146 |
| Potassium<br>*Method: ISE* | 4.7 | mEq/L | 3.5 - 5.5 |
| Chloride | 106 | mEq/L | 96.0 - 109 |

**Interpretation :**

Kidney function tests help to screen the individual for renal disease and to determine the extent or progression of renal disease. These tests also aid in determining drug dosage for drugs excreted through the kidneys. The clinical syndrome resulting from decreased renal function and azotemia is called uremia Renal azotemia: glomerular nephritis and chronic pyelonephritis. Prerenal azotemia: severe dehydration, hemorrhagic shock, and excessive protein intake. Post renal azotemia: urethral stones or tumors and prostatic obstructions Measurement of urea in dialysis fluids is widely used in assessing the adequacy of renal replacement therapy.

In these prerenal situations, the plasma creatinine concentration may be normal.In obstructive post renal conditions, both plasma creatinine and urea concentrations will be increased, although there is often a greater increase in plasma urea than creatinine because of the increased back diffusion.These considerations give rise to the principal clinical utility of plasma urea, which lies in its measurement in conjunction with that of plasma creatinine and subsequent calculation of the urea nitrogen/creatinine ratio. This ratio has been used as a crude discriminator between prerenal and postrenal azotemia. Significantly lower ratios usually denote (1) acute tubular necrosis, (2) low protein intake, (3) starvation, or (4) severe liver disease (decreased urea synthesis). So even though blood urea is not an excellent marker of renal dysfunction as it rises quite late in the dysfunction and its rise is also not exclusive to kidney dysfunction, but for practical purposes serum urea level is still one of the most ordered test and forms an important part of the kidney function test.

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| | | | |
|---|---|---|---|
| **Fasting Plasma Glucose** | **145.89** | mg/dl | Normal : 74 - 106 |
| *Method: Hexokinase* | | | Impaired GT : 107 - 125 |
| | | | Diabetic : >or=126 |

| | | |
|---|---|---|
| > or =126 | Diabetic | Its a group of metabolic disorders characterized by a high blood sugar level over a prolonged period of time |
| 101-125 | Impaired GT | Impaired glucose tolerance means that blood glucose is raised beyond normal levels, but not high enough to warrant a diabetes diagnosis. With impaired glucose tolerance one can face a much greater risk of developing diabetes and cardiovascular disease. |
| 70-100 | Normal | The American Diabetes Association recommends a routine screening for type 2 diabetes. If the results are normal, the screening should be repeated every 3 years. |

| | | | |
|---|---|---|---|
| **Post-Prandial Plasma Glucose** | **244.82** | mg/dl | 80 - 140 |
| *Method: Hexokinase* | | | |

**Interpretation :**
A postprandial glucose reading of 140-199 mg/dl indicates prediabetes.
A postprandial reading over 200 mg/dl indicates diabetes.

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**LIPID PROFILE**

| | | | |
|---|---|---|---|
| Cholesterol-Total | 151.49 | mg/dL | < 200 - Desirable |
| *Method: Spectrophotometry* | | | 200 - 239 -Boderline High |
| | | | > 240 - High |
| Triglycerides level | **177.38** | mg/dL | < 150 - Normal |
| *Method: Serum, Enzymatic, endpoint* | | | 150 - 199 -Boderline High |
| | | | 200 - 499 -High |
| | | | >500 Very -High |
| HDL Cholesterol | 30.3 | mg/dL | < 40 - Low |
| *Method: Serum, Direct measure-PEG* | | | > 40 - Normal |
| LDL Cholesterol | 85.71 | mg/dL | < 100 - Optimal |
| *Method: Enzymatic selective protection* | | | 100 - 129 - Near/Above Optimal |
| | | | 130 - 159 - Borderline high |
| | | | 160 - 189 - High |
| | | | > 190 - Very High |
| VLDL Cholesterol | 35.48 | mg/dL | 6 - 38 |
| *Method: Serum, Enzymatic* | | | |
| CHOL/HDL RATIO | 5.00 | | 0.27 - 5.0 |
| *Method: Serum, Enzymatic* | | | |
| LDL/HDL RATIO | 2.83 | | 2.5 - 3.5 |
| *Method: Serum, Enzymatic* | | | |

**NOTE**
8-10 hours fasting sample is required

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**LIVER FUNCTION TEST (LFT)**

| | | | |
|---|---|---|---|
| TOTAL BILIRUBIN | 0.53 | mg/dl | 0.1 -1.2 |
| *Method: Serum, Jendrassik Grof* | | | |
| DIRECT BILIRUBIN | 0.14 | mg/dL | 0.0 -0.3 |
| *Method: Serum, Diazotization* | | | |
| INDIRECT BILIRUBIN | 0.39 | mg/dl | 0.0 -1.0 |
| *Method: Serum, Calculated* | | | |
| SGPT (ALT) | 6.87 | U/L | 0 - 49 |
| *Method: Serum, UV with P5P, IFCC 37 degree* | | | |
| SGOT (AST) | 12.93 | U/L | 0 - 46 |
| *Method: Serum, UV with P5P, IFCC 37 degree* | | | |
| ALKALINE PHOSPHATASE | 58.70 | U/L | 42 - 125 |
| *Method: DGKC* | | | |
| TOTAL PROTEIN | 7.67 | g/dl | 6.0 - 8.3 |
| *Method: Serum, Biuret, reagent blank end point* | | | |
| SERUM ALBUMIN | 4.55 | g/dl | 3.4 - 5.4 |
| *Method: Serum, Bromocresol green* | | | |
| SERUM GLOBULIN | 3.12 | g/dl | 1.8 - 3.6 |
| *Method: Serum, Calculated* | | | |
| A/G RATIO | 1.46 | | 1.2 - 2.2 |
| *Method: Serum, Calculated* | | | |

**NOTE :**

In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or Non alcoholic fatty liver disease, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

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| | | | |
|---|---|---|---|
| **Total Calcium- Serum** | 10.03 | mg/dl | 8.4 - 10.5 |

*Method : AresenazoIII*

**Hypocalcemia:**
Long-term therapy must be tailored to the specific disease causing the hypocalcemia. The therapeutic endpoint is to achieve a serum calcium level of 8.0 to 8.5 mg/dL to prevent tetany. For symptomatic hypocalcemia, calcium may be administered intravenously

**Hypercalcemia:**
The level at which hypercalcemic symptoms occur varies from patient to patient. Symptoms are common when serum calcium levels are above 11.5 mg/dL, although patients may be asymptomatic at this level. Levels above 12.0 mg/dL are considered a critical value in the Mayo Health System. Severe hypercalcemia (>15.0 mg/dL) is a medical emergency.

**IRON STUDIES**

| | | | |
|---|---|---|---|
| SERUM IRON | **37.0** | ugm/dl | 40 - 160 |
| TIBC | 230 | ugm/dl | 220 - 440 |
| UIBC | 193 | ugm/dl | |
| Transferrin saturation | **16** | % | 20 - 50 |

*Method : Biochemical*

**Test Usage:**
To aid in evaluation of a number of conditions involving red cell production and destruction, iron metabolism, or iron transport.

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## IMMUNOASSAY REPORT

| Test Description | Result | Unit | Biological Reference Range |
|---|---|---|---|
| **Thyroid Function Test (TFT)** | | | |
| T3 | 0.60 | ng/mL | 0.40 - 1.81 |
| T4 | 9.19 | µg/dL | 5.00 - 14.10 |
| TSH(Serum) | 1.07 | µIU/mL | 0.50 - 5.50 |

*Method : ECLIA*

**INTERPRETATION**

| TSH | T3 / FT3 | T4 / FT4 | Suggested Interpretation for the Thyroid Function Tests Pattern |
|---|---|---|---|
| Within Range | Decreased | Within Range | • Isolated Low T3-often seen in elderly & associated Non-Thyroidal illness. In elderly the drop in T3 level can be upto 25%. |
| Raised | Within Range | Within Range | •Isolated High TSHespecially in the range of 4.7 to 15 mIU/ml is commonly associated with Physiological & Biological TSH Variability.<br>•Subclinical Autoimmune Hypothyroidism<br>•Intermittent T4 therapy for hypothyroidism<br>•Recovery phase after Non-Thyroidal illness" |
| Raised | Decreased | Decreased | •Chronic Autoimmune Thyroiditis<br>•Post thyroidectomy,Post radioiodine<br>•Hypothyroid phase of transient thyroiditis" |
| Raised or within Range | Raised | Raised or within Range | •Interfering antibodies to thyroid hormones (anti-TPO antibodies)<br>•Intermittent T4 therapy or T4 overdose<br>•Drug interference- Amiodarone, Heparin,Beta blockers,steroids,anti-epileptics" |
| Decreased | Raised or within Range | Raised or within Range | •Isolated Low TSH -especially in the range of 0.1 to 0.4 often seen in elderly & associated with Non-Thyroidal illness<br>•Subclinical Hyperthyroidism<br>•Thyroxine ingestion" |
| Decreased | Decreased | Decreased | •Central Hypothyroidism<br>•Non-Thyroidal illness<br>•Recent treatment for Hyperthyroidism (TSH remains suppressed)" |
| Decreased | Raised | Raised | •Primary Hyperthyroidism (Graves' disease),Multinodular goitre, Toxic nodule<br>•Transient thyroiditis:Postpartum, Silent (lymphocytic), Postviral (granulomatous,subacute, DeQuervain's),Gestational thyrotoxicosis with hyperemesis gravidarum" |
| Decreased or within Range | Raised | Within Range | •T3 toxicosis<br>•Non-Thyroidal illness |

**References:** 1. Interpretation of thyroid function tests. Dayan et al. THE LANCET • Vol 357 • February 24, 2001
2. Laboratory Evaluation of Thyroid Function, Indian Thyroid Guidelines, JAPI, January 2011,vol. 59

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## URINE EXAMINATION REPORT

| Test Description | Result | Unit | Biological Reference Range |
|---|---|---|---|
| **URINE ROUTINE** | | | |
| **Physical Examination** | | | |
| Colour | Pale Yellow | | Pale Yellow |
| Apperance | Clear | | Clear |
| Specific Gravity | 1.015 | | |
| **Chemical Examination** | | | |
| Blood | Absent | | Absent |
| Sugar | NIL | | Absent |
| Protein | Absent | | Absent |
| Ketone Bodies | Absent | | Absent |
| Bile Salt | Absent | | Absent |
| Bile Pigment | Absent | | Absent |
| Reaction(pH) | 6.0 | | 4.5 - 8.0 |
| **Microscopic Examination** | | | |
| Epithelial Cells | 1-2/hpf | /hpf | 1-2/hpf |
| Pus cells | 2-3/hpf | /hpf | 2-3/hpf |
| RBC's | NIL | /hpf | Nil |
| Casts | Not Seen | | Not Seen |
| Crystals | Absent | | Absent |
| Yeast Cells | Not seen | | Not seen |
| Other Finding | - | | |

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**** End of the report. ****
Please correlate clinically.

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